



PROCEDURE	
CASE MANAGEMENT	
Scope (Staff):	All NMHS Mental Health (MH) Community Staff
Scope (Area):	All NMHS MH Community Areas (excluding Centre for Clinical Interventions (CCI) and Neuroscience Unit (NSU))

1. Aim

All patients and consumers will be allocated a Case Manager from entrance to exit of the Service. All consumers accessing mental health services will be informed about the roles and responsibilities of their Case Manager.

2. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☒	Organisational Development	☒
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☒	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

3. Definitions

Care Coordination: defines a process or approach to reduce fragmentation of service delivery and to ensure care provided by different professionals/agencies is coordinated with the aim of strengthening the collaborative relationships between service providers (organisations) and with consumers, their families and carers. NMHS MH has defined Care Coordination as:

“Care Coordination is consumer driven, person centred and recovery focused. It is tailored to meet the specific needs of the consumer and is the key mechanism required to promote sustained recovery, optimal quality of life and consumer involvement in treatment and care planning decisions. NMHS MH recognises that care coordination must be age appropriate and relevant to the circumstances of consumers and carers.”

Case Management: Case management is a key mechanism to provide collaborative care in partnership with consumers, family members and carers. It is the vehicle for coordinating the care of individual consumers and is the primary method for achieving continuity of care in a timely and effective manner. It is a way of tailoring help to meet individual needs through placing the responsibility of coordinating assessment, care plans and brokering for social support and specialist mental health services with one individual mental health professional identified to work within a multidisciplinary team context and in collaborative partnerships with the consumer, family

members and carers. Care coordination is most effective when the consumer's family, carers and/or significant others are involved in their care, however NMHS MH respects the rights of the consumer to choose not to involve others in their care.

Case management provides opportunities for the consumer to engage in collaborative partnerships with mental health professionals, including the development of a therapeutic relationship with their case manager which can assist in promoting recovery. All case managers in NMHS MH are responsible for involving the consumer and their carer and coordinating services as agreed in the agreed recovery management plan. The complexity of the case manager's roles determined by:

- the number of community agencies involved.
- the complexity and the clinical acuity of the consumer's presentation.
- the range and severity of the consumers unmet needs.

Consumers who have a range of unmet complex needs and clinical acuity and/or poor engagement with mental health services resulting in frequent admissions, are to be referred for Assertive Case Management.

4. Principles

Case Managers will ensure that services provided:

- Are patient centred and trauma informed.
- Are recovery oriented with an emphasis on collaborative care planning.
- Respect diversity and are sensitive to the needs of individuals, families, carers and communities.
- Are culturally sensitive and provide access to interpreter services when required (including AUSLAN) and information in alternative languages where available.
- Respect the needs of Aboriginal people and provide access to Aboriginal Health Workers to with individuals.
- Provide strategies to ensure sexual safety for consumers in accordance with the Chief Psychiatrist's Sexual Safety Guidelines.
- Recognise individual vulnerability and are responsible to the needs of individuals, families, carers and communities.
- Are delivered by clinicians with the range of skill sets required to provide therapeutic interventions and treatments in least restrictive environments in accordance with the requirements of the [Mental Health Act \(MHA\) 2014](#).
- All staff working in Community Mental Health Services are required to adhere to this procedure.

Consumers must be consulted and included in the decision to provide their personal information to individuals, agencies or other services. If there are concerns or request to release personal information, this should be discussed with the consumer and the nominated person, primary care/personal support person, and attempts made to resolve the concerns.

5. Procedure

All consumers referred to and admitted to NMHS MH Community Mental Health services are to have:

1. An identified Case Manager. (See **Appendix 1** for Adult Community pathway to allocate a Case Manager).
2. A comprehensive mental health **assessment**; including a Risk Assessment Management Plan (RAMP) should be completed in line with the [MH Clinical Risk Assessment and Management \(CRAM\) Policy](#) and the [State-wide Standardised Clinical Documentation \(SSCD\) Pathway Chart](#) requirements.
3. A standardised individual **Care Plan** developed:
 - a. In collaboration with the consumer and where appropriate the GP and other stakeholders involved in the care with the multidisciplinary team;
 - b. With the involvement of the consumer's carers, family, or significant others in their care (unless the consumer opts to exclude them);
4. A **Crisis Awareness Plan** is developed where relevant and in collaboration with the consumer and other services or carers involved in care; which identifies early warning signs of relapse and a crisis management plan. A co-produced copy is to be provided to the consumer and any other services (unless inappropriate or refused by the consumer).
5. Every consumer must have a current, comprehensive Client Management and Crisis Plan entered the Psychiatric Service Online Information System (PSOLIS).
6. A Mental Health **Care Transfer Summary** completed to ensure continuity of care across service settings throughout the consumer's journey of care.

All consumers accessing mental health services will be informed of the roles and responsibilities of the case manager and what to expect when they enter the service.

Roles and Responsibilities of a Case Manager

Case Managers may be any member of the Multidisciplinary team who:

- Is competent in delivering mental health care with a clinical skill set required to deliver the teams therapeutic and operational goals.
- Has a clear understanding of the organisational roles in their program.
- Has a good understanding of mental disorders and their management.
- Is able to develop and maintain collaborative partnerships, including therapeutic relationships, with consumers, family members, carers and any other significant other(s).
- Develop joint Recovery Management and Crisis Plans in collaboration with partnered community agencies and consumers where the General Practitioner or Community Managed Organisation may be the lead agency.
- Support consumers who have complex and multiple needs.
- Oversee the delivery of the care set out in the Recovery Management Plan and ensure that it is adhered to by all those involved in delivering care. See **Appendix 2: Standardised Case Manager Pathway for Adult Community MHS**



Case Managers are the Lead Clinician responsible for the co-ordinated care for consumers. Consumers accepted for case management may have complex needs and may require a number of professionals/community services involved in the provision of care.

For continuity of care, consumers who are re-referred to the service may be allocated to their known Case Manager, where it is possible and deemed appropriate.

6. Compliance and Evaluation

NM MH is committed to measurement and evaluation within the broader framework for consumer safety and quality improvement.

NMHS MH will evaluate consumer health outcomes in accordance with the National Outcomes and Casemix Collection (NOCC). The measures that comprise the mental health NOCC include the health of Nations outcome Scales (HoNOS), Life Skills Profile (LSP-16) and the Kessler 10+ (K19+).

NMHS MH (its sites and teams) will measure against an agreed 'performance framework' incorporating measures including safety and quality finance and human resources performance.

NMHS MH participates in the Health Roundtable where it is benchmarked against other Community Mental Health Services.

NM MH will provide opportunities for consumers and carers to provide ongoing feedback on their experience of care through a range of mechanisms such as 'Your Experience of Service' (YES) and Consumer and Carer Advisory Groups.

Related internal policies, procedures and guidelines

[NMHS MH Community Mental Health Services Admission to Discharge Policy](#)
[NMHS MH Inpatient Mental Health Services Admission to Discharge Policy](#)
[NMHS MH Management of Clinical Alerts Policy](#)
[NMHS MH Clinical Record Documentation Procedure](#)
[NMHS MH Code Blue Medical Emergency Procedure - Inpatient MHS](#)
[NMHS MH Code Blue Medical Emergency Procedure - Community MHS](#)
[NMHS MH Medication: Management of High Risk Medication](#)
[NMHS MH Medication: Prescribing Administration and Management for Inpatients Policy](#)
[NMHS MH Medication: Prescribing Administration and Management for Community MHS](#)
[NMHS MH Physical Healthcare Management Policy](#)
[NMHS MH Recognising and Responding to Acute Deterioration Policy](#)
[NMHS MH Safe Use of Low Stimulus Environments Procedure](#)
[NMHS MH Sexual Safety in Inpatient Services Policy](#)
[NMHS MH Person of Concern Protocol](#)
[MHPHDS Mandatory Training Policy](#)
[NMHS Chaperone Policy](#)
[NMHS Clinical Incident Management Policy](#)
[NMHS Emergency Management Policy](#)
[NMHS Clinical Care of People who may be Suicidal Policy](#)

[WA DoH Clinical Care of People Who May Be Suicidal Policy MP0074/17](#)
[WA DoH Clinical Governance, Safety and Quality Policy Framework](#)
[WA DoH Clinical Handover Policy MP 0095](#)
[WA DoH Clinical Handover Guideline](#)
[WA DoH Risk Management Policy](#)
[WA DoH Principles and Best Practice for the Care of People Who May Be Suicidal](#)
[WA DoH Principles and Best Practice for the Clinical Care of People with Mental Health Problems Who May Be At Risk of Becoming Violent or Aggressive](#)
[WA DoH Statewide Standardised Clinical Documentation \(SSCD\)](#)
[WA DoH Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation Guidelines](#)

References

[NMHS Health Point EAP](#)
[WA DoH Community Mental Health Status Assessment; Role of mental health Clinician Policy MP 0099/18](#)
[WA DoH You Matter Guideline](#)
[Mental Health Act 2014](#)
[Chief Psychiatrist's Standards of Clinical Care](#)
[Chief Psychiatrist Standards and Guidelines](#)
[Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services](#)
[MHC Charter of Mental Health Care Principles](#)
[National Standards for Mental Health Services](#)
[National Consensus Statement: Essential Elements for Recognising and Responding to Clinical Deterioration;](#)
[National Consensus Statement Recognising and responding to deterioration in mental State: A scoping review July 2014.](#)

Useful resources (including related forms)

[WA DoH Clinical Alert \(MedAlert\) Policy](#)
[Carer's Recognition Act 2004](#)
[A Practical Guide for Working with Carers of People with a Mental Illness, Consumer Carer Network Australia 2016](#)
[The Guardianship and Administration Act 1990](#)

Policy Sponsor	Co-Director Community Adult NM MH - MHPHDS				
Policy Contact	Co-Director Community Adult NM MH -MHPHDS				
Date First Issued:	12/12/2014	Last Reviewed:	06/02/2023	Review Date:	06/02/2025
Approved by:	Co-Director Community Adult NM MH - MHPHDS			Date:	15/12/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	06/02/2023
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☒ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☒ Std 6: Consumers ☒ Std 7: Carers ☐ Std 8: Governance, leadership and management ☒ Std 9: Integration ☐ Std 10: Delivery of Care ☒ 10.1 Supporting Recovery ☒ 10.2 Access ☒ 10.3 Entry ☒ 10.4 Assessment and Review ☒ 10.5 Treatment and Support ☒ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
12/12/2014	1.0	Adult Program Management	Compiled by: Adult Program (D Bromwell) in consultation with Adult Program Management Team and Area SQRM Committee.
18/02/2014	1.1	Senior Clinical Planning Officer	Minor changes agree via email
24/03/2014	1.2	Senior Clinical Planning Officer	Minor changes agreed via email
12/12/2014	1.3	Senior Clinical Planning Officer	Minor changes agreed via email
14/01/2020	2.0	Coordinator Nursing Staff Development	Major review. Policy aligned with current policies and practice. Revisions include recommended case load allocation, specific references to local policy guidelines for mental health specialist areas
18/05/2020	2.1	A/Policy & Audit Officer	Minor changes agreed via email, addition of hyperlinks to OAMHS Guidelines
06/02/2023	3.0	Co-Director Community Mental Health Services	Scheduled review, procedure updated to reflect current forms and current legislation, information included MH Specialities,

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD 2005).

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Figure 1: Allocation process for a case manager In Adult MH Community Services

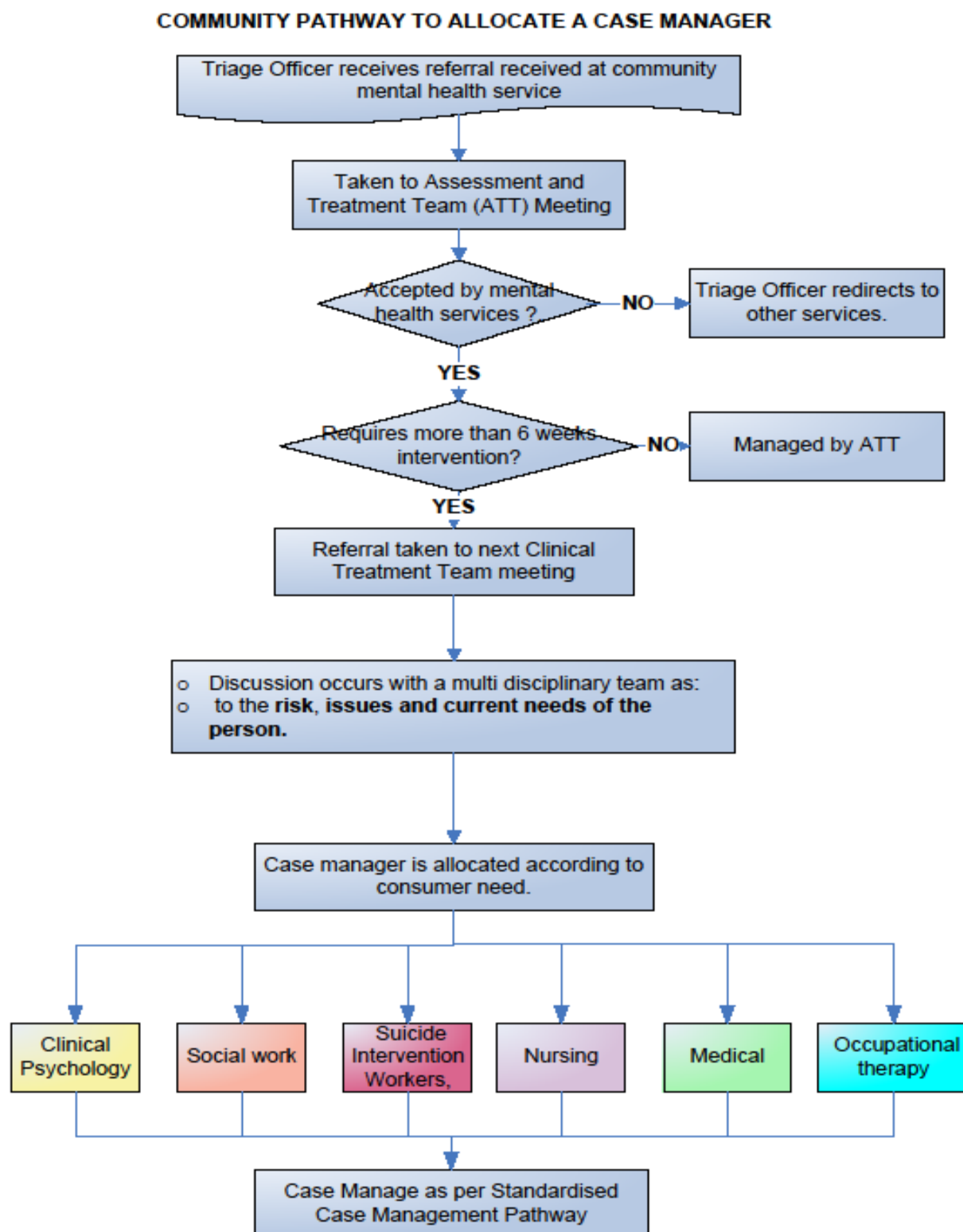


Figure 2: Standardised Adult MHS - Community Case Manager Pathway

